

# WC/2024/227 — Shepherd v Workers' Compensation Regulator

THE QUESTIONS AND THE DOCUMENTS — psychiatric instruction, as it currently stands

Dr Ravikumar Bangalore Krishnaiah, Consultant Psychiatrist, Mind and Memory Service. Source: *01\_Letter\_of\_Instruction\_Report\_B*. Reviewed 14 August 2026.

■ **INTERNAL WORKING DOCUMENT.** Not for service, and not for the practice in this form. Part 3 is working analysis.

## PART 1 — THE QUESTIONS ASKED

Eleven questions. Each stands alone; none combines the clinical question with the statutory one.

### 6.1 Diagnosis, criteria and onset

The diagnosis; the classificatory framework applied (e.g. DSM-5, with the code); **the specific diagnostic criteria met, and when each was met**; and the date of onset as best the clinical record establishes it.

### 6.2 Differential diagnoses

The differential diagnoses considered, and the reasons for including or excluding each.

### 6.3 Psychiatric background

Background psychiatric history — in particular **the entries of 26 October 2022 referring to anxiety and ADHD** (Enclosure E) and any earlier stimulant (Vyvanse) use. Whether there was any relevant pre-existing condition; and if so, **whether the employment matters aggravated it, and to what extent**.

### 6.4 Sources

A **complete list of every document reviewed and every person seen**, with dates and the duration of each examination. Asked for specifically because the completeness of the source list is the first thing tested.

### 6.5 Causation — s 32(1)

On the assumed facts, whether employment — and in particular the rostering and fatigue sequence of 17 March to 1 May 2024 (the consecutive shifts, the admitted 7-hour break, the requests for fatigue relief, the refusal, and the consequent use of his own leave), the conditions of the Switchboard role as described in the assumed facts, and the handling of the concerns raised — was **a significant contributing factor** to the injury with onset pleaded at 18 June 2024. With clinical reasoning, distinguishing the **cause of onset** from factors bearing only on the **subsequent course**.

### 6.6 The April–May 2024 material, and the interval before presentation

What, if anything, he makes of the reports of fatigue recorded in April and May 2024 in the context of the diagnosis made subsequently. Separately, what clinical significance attaches to the interval between pleaded onset (18 June 2024) and first medical presentation — the Hawes certificate recording first attendance for this injury on 1 July 2024.

### 6.7 Competing (non-employment) causes, placed in time

Any non-employment factors relevant to (a) onset at or about 18 June 2024 and (b) subsequent course. For each, **when in time it arose relative to 18 June 2024**. Assumed as facts: employment ceased about October 2024 (later reversed by reinstatement); personal and relationship matters commenced from about December 2024.

### 6.8 Premorbid personality

Whether any premorbid personality features bear on (a) the diagnosis and (b) causation at 6.5 — **from his own clinical assessment**, not by adopting any earlier characterisation.

### 6.9 Mechanism

**Whether, and if so how**, conditions of the kind described in the assumed facts — sustained demand, the degree of control or autonomy, the adequacy of support, and fatigue arising from the rostering — can contribute to a condition of the kind diagnosed.

#### **6.10 Prognosis and capacity**

Prognosis. On capacity, distinguishing (a) capacity to perform the substantive role **with reasonable adjustments** from (b) any current incapacity; and if there is current incapacity, its cause — the condition itself, the consequences of exclusion from the workplace since 3 July 2026, or other factors.

#### **6.11 Change in capacity over time**

How capacity has evolved between the assessment of 13 February 2025 and the present, and the reasons for any change.

## PART 2 — THE DOCUMENTS PROVIDED

Nine enclosures, in three classes. **The class matters more than the content** — only the admitted-facts basket can support an opinion said not to depend on his account.

| Encl. | Document                                                                                                                                                                                                                                                                                                                                                                              | Status                  |
|-------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-------------------------|
| A     | <b>Signed statement</b><br>His own signed statement.<br><b>His account. Signed — but still his.</b>                                                                                                                                                                                                                                                                                   | ASSUMED FACT            |
| B     | <b>Chronology</b><br>A chronology of the relevant events.<br><b>Corroborative of Enclosure A.</b>                                                                                                                                                                                                                                                                                     | ASSUMED FACT            |
| C     | <b>Amended Form 9A (7 April 2026)</b><br>The pleaded case.<br><b>Expressly NOT facts he is asked to assume. Provided so the matters in issue are visible. The case is a course of management conduct as to rostering, fatigue, pay and the handling of concerns — not bullying or harassment.</b>                                                                                     | SCOPE / CONTEXT ONLY    |
| D     | <b>Form 24 admitted-facts extract (18 Feb 2026)</b><br>The Regulator's admissions: the rostered break of 7 hours between the shifts of 17 and 18 March 2024; the criticality of the Switchboard function; the admitted delay in handling.<br><b>■ The only basket the Respondent has conceded. This is what carries the sentence that the opinion does not depend on his account.</b> | ASSUMED FACT — ADMITTED |
| E     | <b>GP records — Our Medical Ashmore</b><br>Including the entries of 26 October 2022 (anxiety, ADHD), earlier Vyvanse use, and the entry of 16 November 2023.<br><b>The 2022 pages are IN and visible. Disclosure is credibility.</b>                                                                                                                                                  | CLINICAL RECORD         |
| F     | <b>Work Capacity Certificate — Dr Peter Hawes</b><br>Records first attendance for this injury on 1 July 2024. Signed 8 September 2024.<br><b>Use for timing. ■ Not for mechanism — the recorded mechanism words are his own account.</b>                                                                                                                                              | CLINICAL RECORD         |
| G     | <b>Certificate of capacity, 3 July 2026</b><br>Certifies him fit for the substantive role with adjustments.                                                                                                                                                                                                                                                                           | CLINICAL RECORD         |
| H     | <b>Report of 13 February 2025</b><br>His own earlier report (MDD diagnosis).<br><b>■ Every page stamped “disclosed for Qsuper and not for medico-legal use”.</b>                                                                                                                                                                                                                      | CLINICAL RECORD         |
| I     | <b>His own clinical records</b><br>The treating file.                                                                                                                                                                                                                                                                                                                                 | CLINICAL RECORD         |

### The three classes

**1 · ASSUMED FACTS (A, B, D)** — the facts he is asked to take as established. ■ Only D is admitted by the Respondent; A and B are the appellant's own.

**2 · SCOPE AND CONTEXT ONLY (C)** — provided to show the matters in issue, expressly not to be assumed.

**3 · CLINICAL RECORD (E–I)** — for clinical review, not as facts about the workplace.

## PART 3 — WHAT REVIEW ON 14 AUGUST FOUND

### ■ 1. The letter still carries the superseded TWO-REPORT structure

Question 6.10 ends: “(The detailed assessment of adjustments is the subject of the separate Metro South report.)” — and section 2 is headed “how it differs from the report for Metro South”.

**There is one report** (settled 6 August, *confirmed-record.md*). If the clinician is told the adjustments are someone else's job, **the report will not contain them** — and RFMI Q6 and Q9 both depend on them. This is the most consequential defect on the page.

**FIX:** delete the parenthesis; rewrite 6.10 to ask for the adjustments in functional terms — predictable rostering; adequate recovery between shifts; not sustained concurrent emergency-code load; day hours; reporting line outside the directorate concerned — and whether each is ordinary and available.

### ■ 2. Review Decision 69983 — three documents, two positions

*confirmed-record.md* (6 Aug) lists it among documents reviewed; the letter-of-instruction verification (9 Aug) removed it entirely as a critical defect; the bundle (12 Aug) calls it the most important document in the pack.

**Establish first: was it put in front of him on 12 August?** Nothing further goes to the practice until that is answered.

### ■ 3. Enclosure D is far smaller than the available admitted record

It is currently the Form 24 extract alone. Available now, with nothing served: the SOFC admissions register of 10 August (the PID admitted in full; the break and “human error”; the COVID attachments “in fact present”; the AVAC chain; the May prodrome); and the **negatives**, now pleadable against the certified list of documents of 14 August.

### ■ 4. Two documents are missing and should be added

**The Chief Executive's letter of 5 June 2026** — no fatigue risk assessment existed; fatigue management implemented only after 30 June 2024; complaints managed solely by email or verbally; **no consequential changes to operating procedures**; MET calls recorded on the shifts of 17–18 March 2024; the leave takings report for 19 March 2024. The employer's own account of the absent controls.

**The AO3 Switchboard role description** — continuous shift work over the full 24-hour period, 7 days a week; Emergency Response duties strictly adhering to protocols and timeframes; call queues to minimum at all times; judgement exercised where procedures are not defined; limited supervision. The employer's own account of the demands.

■ Together these supply demand, control and support from the employer's own hand — and they replace Review Decision 69983 without any of its contamination.

### ■ 5. The single highest-value question is not yet asked

*“Please state whether your opinion at 6.5 would differ if you were to assume **only** the facts at Enclosure D — being facts admitted by the Respondent — and to disregard Enclosure A entirely.”*

If the answer is “no difference”, the opinion cannot be met with the self-report attack and survives any challenge to him as a witness.

### ■ 6. One-report consequence for the schedule

The report goes to Metro South in full and is disclosed in the appeal; it cannot be redacted. **The assumed-facts schedule travels to both audiences.** Admissions, employer documents, dates and arithmetic only — no parallel-track content, no characterisations.